

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 23.8 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 1 Female Ward	Room/Bed: 1 B 1 Female Room/ 1 B 1 Bed 14	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 02-Apr-2023 03:51	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:07

PROVISIONAL DIAGNOSIS:

21 YR/PRIMI/41 WEEK POG/THROMBOCYTOPENIA(PLT-56K)

FINAL DIAGNOSIS:

21 YR/P1L1/POD 2 OF EMERGENCY LSCS I/V/O GRADE 2 MSL

OPERATION PERFORMED:

EMERGENCY LSCS I/V/O GRADE 2 MSL IN EARLY LABOUR On 02-Apr-2023

SURGEONS: DR TULIKA

ASSISTANT SURGEONS/SENIOR RESIDENTS: DR CHETAN, DR ANUSHKA

OPERATIVE FINDINGS: UNDER ADEQUATE SPINAL ANAESTHESIA, PFANNENSTIAL INCISION GIVEN. ABDOMEN OPENED IN LAYERS LUS WAS IDENTIFIED, A SMALL NICK WAS GIVEN AT LUS AND EXTENDED BILATERALLY. LIQUOR WAS CLEAR AND ADEQUATE AMOUNT WAS NOTED. TERM/ALIVE /MALE BABY WEIGHING 3.315KG AT 5:44 PM ON 02/04/23 . BABY CRIED IMMEDIATELY AFTER BIRTH. PLACENTA AND MEMBRANES DELIVERED COMPLETELY. B/L TUBES AND OVARIES WERE NORMAL. INTRAOP BLOOD LOSS- 700 ML. BABY WAS BREASTFED WITHIN 1 HOUR AFTER DELIVERY

CHIEF COMPLAINTS:

21 YR/PRIMI/41 WEEK POG/THROMBOCYTOPENIA(PLT-56K) WITH CHIEF COMPLAIN OF PAIN IN ABDOMEN SINCE 1 AM AND BLEEDING PV SINCE 10 AM OF 1.4.2023

HISTORY OF PRESENT ILLNESS: 21 YR/PRIMI/41 WEEK POG/THROMBOCYTOPENIA(PLT-56K) WITH CHIEF COMPLAIN OF PAIN IN ABDOMEN SINCE 1 AM AND BLEEDING PV SINCE 10 AM OF 1.4.2023.. ON CLINICAL EXAMINATION, SHE WAS VITALLY STABLE, PERCEIVEING FETAL MOVEMENTS WELL, HER NST WAS REACTIVE AT THE TIME OF ADMISSION.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
03-Apr-2023 11:43	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.01	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	1.26	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	1.14	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	11.14	2 - 7 x10 ³ /μL

02-Apr-2023 16:44

Basophil	0.1	00 - 01 %
Eosinophils	0.1	01 - 06 %
HCT	35.1	36 - 46 %
Hemoglobin	10.5	12 - 15 g/dl
Lymphocytes	9.3	20 - 40 %
MCH	25.1	27 - 32 pg
MCHC	29.9	31.5 - 34.5 g/dL
MCV	83.8	83 - 101 fL
Monocytes	8.4	2 - 10 %
Neutrophils	82.1	40 - 80 %
Platelet Count	120	150 - 410 x10 ³ /μL
RBC	4.19	3.8 - 4.8 x10 ⁶ /μL
Red Cell Distribution Width (RDW CV)	16.6	11.6 - 14.0 %
Total Leukocyte Count(TLC)	13.56	4 - 10 x10 ³ /μL
Comments on PBS	Refer Reports*	-
Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
Absolute Eosinophil Count	0.05	0.02 - 0.5 x10 ³ /μL
Absolute Lymphocyte Count	2.30	1 - 3 x10 ³ /μL
Absolute Monocyte Count	1.10	0.2 - 1.0 x10 ³ /μL
Absolute Neutrophil Count	10.36	2 - 7 x10 ³ /μL
Basophil	0.1	00 - 01 %
Eosinophils	0.4	01 - 06 %
HCT	40.5	36 - 46 %
Hemoglobin	11.6	12 - 15 g/dl
Lymphocytes	16.6	20 - 40 %
MCH	25.3	27 - 32 pg
MCHC	28.6	31.5 - 34.5 g/dL
MCV	88.4	83 - 101 fL
Monocytes	8.0	2 - 10 %
Neutrophils	74.9	40 - 80 %
Platelet Count	75	150 - 410 x10 ³ /μL
RBC	4.58	3.8 - 4.8 x10 ⁶ /μL
Red Cell Distribution Width (RDW CV)	17.1	11.6 - 14.0 %
Total Leukocyte Count(TLC)	13.82	4 - 10 x10 ³ /μL
APTT HEADER	23.0	23.2 - 32.4 sec
APTT NOTE	Refer Reports*	-

02-Apr-2023 04:05

ACTIVATED PARTIAL
THROMBOPLASTIN
TIME (APTT)

	clinical details	--	-
BLOOD GROUPING	Blood Group	A	-
ABO and RhD	Note	Refer Reports*	-
	RH Type	Positive	-
COMPLETE BLOOD	Comments on PBS	Refer Reports*	-
COUNT (CBC)			
	Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
	Absolute Eosinophil Count	0.05	0.02 - 0.5 x10 ³ /μL
	Absolute Lymphocyte Count	2.37	1 - 3 x10 ³ /μL
	Absolute Monocyte Count	0.87	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	10.11	2 - 7 x10 ³ /μL
	Basophil	0.1	00 - 01 %
	Eosinophils	0.4	01 - 06 %
	HCT	38.1	36 - 46 %
	Hemoglobin	11.3	12 - 15 g/dl
	Lymphocytes	17.7	20 - 40 %
	MCH	24.7	27 - 32 pg
	MCHC	29.7	31.5 - 34.5 g/dL
	MCV	83.2	83 - 101 fL
	Monocytes	6.5	2 - 10 %
	Neutrophils	75.3	40 - 80 %
	Platelet Count	62	150 - 410 x10 ³ /μL
	RBC	4.58	3.8 - 4.8 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	16.6	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	13.41	4 - 10 x10 ³ /μL
HBsAg (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
HCV Antibody (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
LACTATE	LDH	182	230 - 460 u/l
DEHYDROGENASE (LDH)			
Liver Fuction Test (Lft)	Albumin	3.6	4.0 - 5.0 gm/dL
	ALP	389	44 - 107 U/L
	ALT	15	7 - 41 U/L
	AST	30	12 - 38 U/L
	Direct Bilirubin	0.21	0.1 - 0.4 mg/dl
	globulin	3.6	2.0 - 3.5 gm/dL



	Indirect Bilirubin	0.98	0.20 - 0.90 mg/dl
	Total Bilirubin	1.19	0.3 - 1.3 mg/dl
	Total Protein	7.2	6.7 - 8.6 gm/dL
PLATELET COUNT	Footer		-
	Comments on PBS	62	150 - 410 x10 ³ /μL
PROTHROMBIN TIME	clinical details	--	-
(PT INR)			
	International Normalised	0.9	-
	Ratio (INR)		
	PT INR Header	8.7	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: BLOOD GRP-A POSITIVE

USG(26/12/2022)-SINGLE LIVE INTRAUTERINE FETUS IN CEPHALIC PRESENTATION CORRESPONDING TO 28 WEEK 3DAY.USG EDD-17/3/2023

HOSPITAL COURSE: 21 YR/PRIMI/41 WEEK POG/THROMBOCYTOPENIA(PLT-56K) WITH CHIEF COMPLAIN OF PAIN IN ABDOMEN SINCE 1 AM AND BLEEDING PV SINCE 10 AM OF 1.4.2023.. ON CLINICAL EXAMINATION, SHE WAS VITALLY STABLE, PERCEIVEING FETAL MOVEMENTS WELL, HER NST WAS REACTIVE AT THE TIME OF ADMISSION.CONDITION AT ADMISSION-

GENERAL EXAMINATION:

PR: 78/MIN, BP: 110/70 MMHG, RR: 14/MIN, TEMP: AFEBRILE

PALLOR/ ICTERUS/CYANOSIS/CLUBBING/ EDEMA- ABSENT

SYSTEMIC EXAMINATION: CVS: S1 S2 , RS- B/L AIR ENTRY

P/A- UTERUS 32 WEEK,CEPHALIC,FHR-138/MIN,SFH-33 CM

P/S- OS OPEN. SHOW

P/V- CERVIX SOFT, CENTRAL OS,IFL 50% EFFACED VERTEX PPT,MEMBRANE ,PELVIS ADEQUATE

SHE WAS TRANSFUSED WITH 4 UNITS OF RDP AND CONSENT FOR INDUCTION OF LABOUR WAS TAKEN.. STRICT FETOMATERNAL SURVELLIENCE WAS DONE.POST INDUCTION WAS NON REACTIVE AND THE PATIENT WAS TAKEN FOR EMERGENCY LSCS I/V/O GRADE 2 MSL IN EARLY LABOUR.SHE DELIVERED A TERM ALIVE MALE WEIGHING 3.315KG AT 5:44 PM ON 02/04/23 BY VERTEX PRESENTATION. BABY CRIED IMMEDIATELY AFTER BIRTH.PLACENTA AND MEMBRANE DELIVERED COMPLETELY. , EPISIOTOMY WAS REPAIRED. NO INTRAPARTUM COMPLICATIONS NOTED. MOTHER AND BABY PASSED STOOL AND URINE. BOTH MOTHER AND BABY ARE FAIR AT PRESENT HENCE BEING DISCHARGED WITH ADVISE TO FOLLOWUP IN GYNEAC OPD.

MODE OF DELIVERY: EMERGENCY LSCS I/V/O GRADE 2 MSL IN EARLY LABOUR

DATE OF DELIVERY: 2/4/2023

OUTCOME: A TERM ALIVE MALE WEIGHING 3.31KG AT 5:44 PM ON 02/04/23 BY VERTEX PRESENTATION

DELIVERY DETAILS: PATIENT IN DORSAL LITHOTOMY POSITION. WITH GOOD UTERINE CONTRACTION.SHE DELIVERED A TERM ALIVE MALE WEIGHING 3.315KG AT 5:44 PM ON 02/04/23 BY VERTEX PRESENTATION.BABY CRIED IMMEDIATELY AT BIRTH. CORD CLAMPED AND CUT AND BABY HANDED OVER TO PEDIATRICIAN. PLACENTA AND MEMBRANES DELIVERED OUT COMPLETELY. EPISIOTOMY WOUND WAS REPAIRED. NO INTRAPARTUM AND POSTPARTUM COMPLICATIONS NOTED. BABY WAS BREASTFED WITHIN 1 HR AFTER BIRTH .



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



CONDITION AT DISCHARGE: GENERAL EXAMINATION:

PR: 78/MIN, BP: 110/70 MMHG, RR: 14/MIN, TEMP: AFEBRILE

PALLOR/ ICTERUS/CYANOSIS/CLUBBING/ EDEMA- ABSENT

SYSTEMIC EXAMINATION: CVS: S1 S2 , RS- B/L AIR ENTRY

P/A- SOFT, NON TENDER, UTERUS CONTRACTED.

L/E: NO ACTIVE BLEEDING, LOCHIA HEALTHY

B/L BREASTS: SOFT, NON TENDER

STOOL AND URINE PASSED.

ADVICE ON DISCHARGE:

FOR MOTHER:

- HIGH PROTEIN DIET
- TAB. CEFIXIME 200 MG BD FOR 2 DAYS
- TAB. PANTOP 40MG OD ES FOR 2 DAYS
- TAB. PCM 500MG PO TDS X 2 DAYS F/B SOS IF PAIN
- T. IRON FOLIC ACID OD FOR 3 MONTHS
- T. CALCIUM VITAMIN D BD FOR 6 MONTHS
- MAINTAIN PERINEAL HYGIENE
- PRACTICE KEGEL EXERCISES 2 SETS OF 25 EACH TWICE DAILY.
- SEXUAL ABSTINENCE FOR 6 WEEKS.
- AVOID SQUATTING, STRAINING, CROSSED LEG SITTING.
- HOME ISOLATION, WEAR MASK, SOCIAL DISTANCING, NO SOCIAL GATHERING, BE AT HOME.

FOR BABY

- 1) EXCLUSIVE BREASTFEEDING AND BURPING AFTER EACH FEED
- 2) IMMUNISATION AS PER SCHEDULE
- 3) CORD CARE/EYE CARE
- 4) FOLLOW UP AS PER ADVICE OF PEDIATRICIAN

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 10-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW IN GYNEC OPD AFTER 10 DAYS FOR SUTURE REMOVAL ON MONDAY/FRIDAY OR SOS IN CASE OF BLEEDING PV, SEVERE PAIN ABDOMEN, FOUL SMELLING DISCHARGE AND/OR FEVER, AND/OR BREAST PAIN.

REVIEW IN AYUSH COVID-OPD I/C/O COLD ,COUGH, FEVER, BREATHING DIFFICULTY , BODY PAINS AND/OR DIARRHEA.

Summary Prepared By

anonymous

Summary Approved By

anonymous